A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

14 May 2026

Dr Claire Veith
East Brunswick Medical Centre

Dear Dr Veith,

Re: Ms Louise Crowe
DOB: 02/06/1963 Phone: 0412 442 280
Address: 7 Herbert Street, PRESTON, VIC, 3072
Email: louise.crowe@loretotoorak.vic.edu.au

Summary

- **Normal gastroscopy and colonoscopy.**
- **Symptomatic improvement with reduced stress and pelvic floor physiotherapy.**
- **For increased fibre supplementation to manage stool consistency.**
- **No action required by you.**

I had the pleasure of reviewing Louise Crowe following her gastroscopy and colonoscopy, which were recently performed via the public waitlist. You will be pleased to know that the investigations did not reveal any significant pathology.

The gastroscopy identified benign fundic gland polyps, which require no further intervention. The colonoscopy was clear of polyps or malignancy, showing only typical internal haemorrhoids. Biopsies taken from both procedures were entirely normal, specifically excluding coeliac disease, microscopic colitis, and H. pylori infection.

Louise reports a significant improvement in her gastrointestinal symptoms, which appears to correlate with her being on long service leave and the associated reduction in work-related stress. While she previously experienced bowel accidents and urgency—particularly in the context of her work as a teacher—she currently reports much better control. She is currently opening her bowels twice daily. We also discussed her significant pelvic floor dysfunction. She is currently managing a vaginal prolapse and is engaged with pelvic floor physiotherapy. Given her obstetric history of four vaginal deliveries, I believe this physiotherapy is a critical component of her ongoing management.

Impression and Plan The current improvement in Louise's symptoms is likely a combination of reduced stress and her commitment to pelvic floor exercises. To further improve stool consistency, I have suggested she optimize her fibre intake. She found Metamucil of limited benefit at a low dose, so I have recommended she either increase the dose or trial alternative supplements such as K Fiber or partially hydrolysed guar gum (PHGG).

I do not believe any new medications are required at this stage while she is symptomatic. However, I have discussed potential strategies for when she returns to the classroom, including the "as needed" use of loperamide or the future consideration of a neuromodulator to address the stress-gut axis if urgency becomes

All appointments and enquires
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Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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a prominent issue again. I have not scheduled a formal follow-up appointment, but I have asked Louise to contact my rooms should her symptoms deteriorate.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

Investigation Results:

CROWE, LOUISE : SVH 937484

DOB: 02/06/1963 Age: 62 years

7 Herbert Street, Preston 3072

Results from 30/04/2026 ..... Episode# 26P317062 Test Hist (Histology - Routine)

FINAL ANATOMICAL PATHOLOGY AAnat

Lab #: 2026B011296 SUPPLEMENTARY REPORT H. pylori immunohistochemical stain performed on specimen 3 and is negative. Pathologist: Dr Simin Daneshvar **CLINICAL NOTES** Dysphagia, diarrhoea. **MACROSCOPIC DESCRIPTION** 1. Specimen labelled "Mid Oesop bx". A mucosal fragment measuring 2-3 mm. A1. 2. Specimen labelled "Lower Oesop bx". Two mucosal fragments measuring 2-3 mm. A1. 3. Specimen labelled "Gastric Antrum bx ". Four mucosal fragments measuring 2-3 mm. A1. 4. Specimen labelled "Duo bx". Three mucosal fragments measuring 2-3 mm. A1. 5. Specimen labelled "Random Colon bx". Four mucosal fragments measuring 2-3 mm. A1. **MICROSCOPIC DESCRIPTION** 1. The sections consist of superficial squamous mucosa with no underlying stroma. No significant basal zone hyperplasia is seen and infrequent number of intraepithelial lymphocytes and eosinophils are present. Neutrophils are not seen. Fungal elements are not identified in the H&E stained sections. There is no dysplasia. Glandular mucosa is not included. 2. There is superficial squamous mucosa with reactive change and scattered intraepithelial lymphocytes. Eosinophils are infrequent and neutrophils are not seen. Negative for dysplasia. Glandular mucosa is not seen. 3. There is non-specialised gastric mucosa with reactive change. There are scattered chronic inflammatory cells within the lamina propria. Negative for intestinal metaplasia, neutrophilic inflammation, Helicobacter-like organisms or granuloma. Negative for dysplasia. 4. The sections consist of small intestinal mucosa with preserved villous architecture. There is a normal complement of chronic inflammatory cells within the lamina propria. The numbers of intraepithelial lymphocytes are within normal limits. No neutrophils, granulomata, parasites, or dysplasia identified. 5. There is colonic mucosa with relatively preserved crypt architecture. There is focal mild increase in the numbers of chronic inflammatory cells within the lamina propria. Negative for neutrophilic inflammation, granuloma or dysplasia. Features are not diagnostic of microscopic colitis. **PATHOLOGIST'S OPINION** **1. Mid oesophageal biopsies - Superficial squamous mucosa within normal limits. 2. Lower oesophageal biopsy - Superficial squamous mucosa with focal mild reactive change. 3. Gastric antrum biopsy - Mild chronic inactive**

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gastritis. COMMENT: H.pylori immunostain will be performed and a supplementary report issued when results become available. 4. Duodenal biopsy - Within normal limits. 5. Random colon biopsy - Mild nonspecific change. Negative for active inflammation. Pathologist: Dr Simin Daneshvar SNOMED T#: 67 SNOMED M#: 00100

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Chamara Basnayake at 14/05/26 07:53

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